

Hair Loss on GLP-1

*What's Really Happening and How to
Minimize It*

Disclaimer

This book is for educational and informational purposes only. It is not medical advice and does not constitute a recommendation to start, stop, or change any medication or treatment. GLP-1 receptor agonists are prescription medications that should only be used under the supervision of a qualified healthcare provider.

The nutritional guidance, exercise information, and lifestyle strategies in this book are general wellness information and are not tailored to your individual medical situation. Always consult your doctor, registered dietitian, or other qualified health professional before making changes to your diet, exercise routine, or medication management.

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Why Hair Loss Happens on GLP-1

Hair loss is one of the more distressing side effects of GLP-1 treatment — partly because it's not immediately obvious why a weight loss medication would cause hair to shed, and partly because hair loss carries significant psychological weight for most people.

Telogen Effluvium — The Mechanism

The hair loss most commonly reported by GLP-1 users is telogen effluvium — a form of temporary, diffuse shedding triggered by a significant physiological stress. It's the same type of hair loss that occurs after major surgery, severe illness, childbirth, extreme emotional stress, and any other significant metabolic disruption.

Hair follicles cycle through active growth (anagen), transition (catagen), and resting/shedding (telogen) phases. Significant physiological stress pushes a larger-than-normal proportion of follicles simultaneously into the telogen phase. The shedding, which occurs as these resting follicles release the hair shaft, typically begins two to three months after the

triggering event — which is why people on GLP-1 often don't experience hair loss until several months into treatment.

The good news about telogen effluvium: it is self-limiting. The follicles are not permanently damaged. When the triggering stress resolves — which for GLP-1 means when the body adapts to the new weight and eating pattern — the follicles return to normal cycling and the hair regrows, usually within three to six months of the shedding stopping.

What Makes It Worse

Rapid weight loss is the primary trigger. The faster the weight is lost, the more physiological stress the body experiences, and the more pronounced the telogen effluvium. This is why GLP-1 users on faster-escalating doses or with higher initial rates of weight loss often report more hair loss.

Protein deficiency significantly worsens hair loss. Hair is made primarily of keratin — a protein. When protein intake is inadequate, the body deprioritises protein for hair growth in favour of maintaining organ function and lean tissue. Inadequate protein during GLP-1-assisted weight loss compounds the telogen effluvium with protein-deficiency hair loss.

Micronutrient deficiencies — particularly iron, zinc, and biotin — also contribute. These are common on reduced calorie intakes and all play roles in healthy hair follicle function.

"The hair will come back. The follicles aren't gone. But there are things you can do to reduce the shedding."

Minimising Hair Loss on GLP-1

You cannot entirely prevent telogen effluvium during significant weight loss. You can meaningfully reduce its severity.

Protein — The Most Important Intervention

Meeting protein targets is the single most important thing you can do to minimise hair loss on GLP-1. A minimum of 1.2–1.6 grams of protein per kilogram of body weight per day provides the amino acids required for hair follicle function. In practice, this means prioritising protein at every meal and supplementing with protein shakes if food intake is insufficient to reach targets.

Collagen peptides deserve specific mention in the context of hair loss. Collagen is rich in the amino acids glycine, proline, and hydroxyproline — precursors for keratin synthesis. While the evidence for collagen supplementation specifically for GLP-1 hair loss is limited, the general evidence for hair, skin, and nail structure is reasonable, and the side effect profile is excellent.

Targeted Micronutrients

Iron: have your ferritin checked if you're experiencing significant hair loss. Ferritin (stored iron) can be in the normal reference range while still being insufficient for optimal hair growth. Most hair loss specialists recommend a ferritin level above 70 ng/mL for hair growth support, though the standard 'normal' range starts at 12–15.

Zinc: a cofactor for multiple enzymes involved in hair growth. Common in meat, shellfish, legumes, nuts, and seeds. Supplementation is appropriate if dietary intake is low or blood levels are below optimal.

Biotin: widely marketed for hair growth. The evidence for biotin supplementation in people without a biotin deficiency is actually limited. However, it is safe and inexpensive, and GLP-1 users may have reduced biotin intake from reduced food variety. 5000mcg per day is the typical supplement dose.

Stress Management

Chronic psychological stress independently increases cortisol, which promotes telogen effluvium. The nervous system regulation practices throughout this series — breathwork, movement, sleep — all contribute to stress management and thus, indirectly, to hair health during the weight loss period.

"Eat enough protein. Check your iron. Be patient. The hair comes back."

Scalp Care During Shedding

Practical scalp and hair care that minimises the appearance of shedding and supports follicle health.

Hair Care Adjustments

Gentle handling reduces mechanical hair loss on top of telogen effluvium shedding. Avoid tight hairstyles (ponytails, braids) that put traction on the follicle. Use a wide-tooth comb rather than a brush on wet hair. Minimise heat styling. These adjustments don't change the underlying telogen effluvium, but they reduce the additional shedding from physical manipulation.

Scalp massages with a scalp massager or fingertips have reasonable evidence for stimulating blood flow to follicles and may modestly support regrowth. They're also pleasant and stress-reducing, which has secondary benefits.

When to See a Dermatologist

If hair loss is severe, patterned (not diffuse), accompanied by scalp changes, or not improving six

months after weight loss has stabilised, a dermatologist can assess for other causes — including androgenetic alopecia, thyroid disorders, and other conditions that require specific treatment.

Telogen effluvium from GLP-1 typically resolves without medical intervention once the triggering stress reduces. Hair loss that is persistent or progressive beyond this warrants professional evaluation.

"Temporary is the key word. Manage it, support it, and let time do the rest."
